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ABSTRACT

Retention in Care Among PLHIV Accessing Care at Public Treatment Sites in Jamaica

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Objectives: To characterize Jamaica's HIV population based on retention status, identify factors associated with retention in care, and develop recommendations to improve retention.

Methods: A retrospective analysis was conducted using data from the Treatment Site Information System (TSIS). A total of 40,272 PLHIV were identified, with exclusions for duplicates, transfers, non-regional health authority attendees, and deaths, leaving 12,542 for analysis. Retention was defined as active HIV care engagement without missed appointments >90 days, while loss to follow-up (LTFU) was >90 days without a clinic visit. Retention status was compared across demographic, socioeconomic, and clinical variables using descriptive statistics and logistic regression to identify factors significantly associated with retention.

Results: Overall, 83% (n=10,466) were retained and 17% (n=2,076) were LTFU. Retention was significantly higher among females (84% vs. 82%, p=0.003) and married individuals (OR=2.0, p<0.001). Retention was lowest in PLHIV aged 20–44 years compared to those ≥50 (p<0.001). PLHIV receiving care outside their parish of residence were less likely to be retained (OR=0.7, p<0.001). Receiving SMS reminders was strongly associated with retention (OR=20.1, p<0.001). Clinical factors associated with higher retention included first CD4 <200 (OR=1.9, p<0.001) and viral load suppression, with undetectable VL increasing retention fivefold (OR=5.4, p<0.001).

Conclusion: Retention in HIV care at public sites in Jamaica is generally high but suboptimal among younger age groups, males, and those accessing care outside their parish. Strengthening SMS reminder systems, targeted interventions for vulnerable groups, and expanded service access are recommended to improve retention.

Keywords: HIV, retention in care, Jamaica, PLHIV, TSIS, lost to follow-up

The abstract count is 249 words without a title or authors, or keywords.

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